

Task 5 – Individual Case Safety Report (ICSR)

Pharmacovigilance Internship Submission | Mock ICSR

Case ID: ICSR-001

1. Case & Patient Information

Report Type	Initial report
Date Received	August 9, 2026
Source	Treating physician
Patient	Male, 56 years
Weight	78 kg
Medical History	Osteoarthritis of right knee; controlled hypertension; no known GI bleeding; no known drug allergies
Reporter	Treating physician, healthcare professional
Reporter Contact	Not provided

2. Suspected Drug

Drug	Diclofenac
Dose	50 mg
Route	Oral
Frequency	Twice daily
Indication	Osteoarthritis-related knee pain
Therapy Start	August 1, 2026
Therapy Stop	August 8, 2026
Action Taken	Drug discontinued

3. Concomitant Medicines

Drug	Dose	Route	Indication	Duration
Amlodipine	5 mg once daily	Oral	Hypertension	3 years
Paracetamol	500 mg as needed	Oral	Knee pain	Occasional use for 6 months

4. Adverse Event & Investigations

Symptoms	Black, tarry stools, dizziness and general weakness
Onset	August 8, 2026
Hospitalization	Admitted August 8, 2026
Laboratory Findings	Hemoglobin 8.9 g/dL; stool occult blood positive
Endoscopy	Gastric ulcer with active bleeding
Clinical Diagnosis	Acute upper gastrointestinal bleeding associated with gastric ulcer

5. Management & Outcome

Management	Diclofenac discontinued; IV proton-pump inhibitor, fluids and endoscopic treatment
Blood Transfusion	Not required
Outcome	Recovered; melena, dizziness and weakness resolved; hemoglobin stabilized
Discharge	August 12, 2026
Follow-up	August 20, 2026: clinically stable

6. ICSR Validity Assessment

Identifiable Patient	Yes
Identifiable Reporter	Yes – treating physician
Suspected Product	Yes – diclofenac
Adverse Event	Yes – confirmed bleeding gastric ulcer
Minimum Criteria	All met
ICSR Validity	Valid

7. MedDRA Coding – Five-Level Hierarchy

MedDRA has five hierarchical levels: SOC, HLGT, HLT, PT and LLT. For this mock case, the confirmed clinical diagnosis is coded using the most specific documented concept.

SOC	Gastrointestinal disorders
HLGT	Gastrointestinal ulceration and perforation
HLT	Gastric ulcers and perforations
PT	Gastric ulcer haemorrhage — 10017826
LLT	Gastric ulcer haemorrhage — identical LLT associated with the PT

8. Seriousness, Severity & Expectedness

Seriousness	Serious
Seriousness Criterion	Inpatient hospitalization
Severity	Severe
Expectedness	Expected for this mock case
Clinical Impact	Significant
Rationale	Hospitalization and endoscopic treatment occurred. The mock reference safety information states that GI ulceration and bleeding are established diclofenac reactions.

9. Causality Assessment – Naranjo Algorithm

The Naranjo scale contains 10 questions. A total score of 5–8 is interpreted as Probable.

#	Question	Answer	Score
1	Previous conclusive reports?	Yes	+1
2	Event appeared after suspected drug?	Yes	+2
3	Improved after discontinuation?	Yes	+1
4	Reappeared on re-administration?	Do not know	0
5	Alternative causes present?	No	+2
6	Reappeared with placebo?	Do not know	0
7	Toxic drug concentration documented?	Do not know	0
8	Dose-response relationship?	Do not know	0
9	Similar reaction with same/similar drug previously?	No	0
10	Confirmed by objective evidence?	Yes	+1

Score calculation: +1 +2 +1 +0 +2 +0 +0 +0 +0 +1 = 7

Final Naranjo classification: Probable (5–8)

Rationale: The event followed diclofenac exposure, improved after withdrawal, no alternative drug-related trigger was identified in the mock case, and the gastric ulcer with active bleeding was objectively demonstrated by endoscopy. Rechallenge and toxic drug levels were not available.

10. Final Assessment

ICSR Validity	Valid
Suspected Drug	Diclofenac
Primary MedDRA PT	Gastric ulcer haemorrhage (10017826)
Seriousness	Serious – hospitalization
Severity	Severe
Expectedness	Expected
Causality	Probable – Naranjo 7
Action Taken	Diclofenac discontinued
Outcome	Recovered

11. ICSR Narrative

A 56-year-old male weighing 78 kg with osteoarthritis of the right knee and controlled hypertension was started on diclofenac 50 mg orally twice daily on August 1, 2026 for osteoarthritis-related knee pain. Concomitant medicines were amlodipine 5 mg once daily and occasional paracetamol 500 mg.

On August 8, 2026, after seven days of diclofenac treatment, the patient developed black, tarry stools, dizziness and general weakness and was admitted to hospital. Hemoglobin was 8.9 g/dL and stool occult blood was positive. Upper GI endoscopy demonstrated a gastric ulcer with active bleeding.

Diclofenac was discontinued. The patient received IV proton-pump inhibitor therapy, fluid replacement and endoscopic treatment. No transfusion was required. The patient improved and was discharged on August 12, 2026. At follow-up on August 20, 2026, he remained clinically stable.

The case was assessed as a valid ICSR. The primary MedDRA concept was Gastric ulcer haemorrhage. The event was serious because of hospitalization and severe in intensity. It was considered expected for this mock case based on the stated safety information. Naranjo scoring gave a total of 7, supporting a Probable relationship with diclofenac.

12. Key Learning

This mock ICSR demonstrates ICSR validity assessment, clinical event identification, five-level MedDRA hierarchy selection, seriousness/severity/expectedness assessment, Naranjo causality assessment, and chronological narrative writing.